



A SOOTHEMADE BOOK

Nine Months You Are Also Living

A pregnancy companion for the partner.

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the partner at the early scan, watching the screen, not knowing whether to cry from joy or from how scared you are.

You are also in this pregnancy. You are also doing something.

A note from Maya

This book is for you.

Not for your pregnant partner. For you, the non-birthing partner, the one who is also in this pregnancy but in a different body and a different chair.

Almost nobody writes for you. The friends ask about your partner. The provider talks to your partner. The appointments are about your partner's body. Your role is real, your fear is real, your love for the small person who has not arrived yet is real, and nobody has handed you a playbook.

This is the playbook.

It is written assuming you are showing up. You will not need every page. The pages you do need will be there.

With care, Maya

This book is not medical advice. For specific questions about the pregnancy or your partner's health, the provider is the right person. Partner mental health is real. The crisis-line page is at the back.

Chapter 1: You Are Also Doing Something

A chapter to come back to.

What is commonly assumed about you:

- That you are excited.
- That you are supportive by default.
- That you will figure it out.
- That you do not need a playbook.
- That you do not have your own fears.
- That you can absorb the changes to your partner without changes to yourself.

What is actually true.

You are also in this pregnancy. You are also scared sometimes. You are also grieving the version of your life before. You are also bonding with a person you have not met. You are also, sometimes, doing the wrong thing and not knowing.

The things you are doing that no one is counting.

- Driving to the appointments.
- Researching the things she is too tired to research.
- Holding her hair back when she throws up.
- Eating food she cannot stand to smell, in the other room.
- Saying nothing when she is irritable for no reason you can see.
- Picking up the financial slack she might be missing.

- Doing the emotional labor she is currently too depleted for.
- Reading this book at 11pm so you can do better tomorrow.

You are doing real work. Even if nobody at the dinner party asks.

Chapter 2: Trimester One, What Is Happening for Your Partner

Most pregnant people in the first trimester are more tired than they have ever been. Often nauseated, sometimes severely. Experiencing sudden food aversions (often to things they previously loved). Hormonally adjusting in ways that affect mood, sleep, energy. Scared (early-pregnancy loss is statistically real). Keeping a secret from most of the world. Sometimes unable to perform their normal work, social, or household role.

What is happening that you cannot see:

- Implantation, organ development, the beginnings of brain.
- Hormone levels rising fast.
- Body diverting massive amounts of energy to building.
- Often a feeling of being "outside" of one's own life.

A note on severity.

"Morning sickness" is misleading. Many people are sick all day. A small portion experience hyperemesis gravidarum (severe sickness that requires medical management).

If your partner cannot keep any food or liquid down for more than a day, or is losing significant weight, this is a call to the provider, not a "ride it out" situation.

Chapter 3:

Trimester One,

How to Be Useful

For the exhaustion. Pick up more of the household load without being asked. Let her nap on weekends without making her feel guilty. Take care of meals. Take over the dog walks if you have a dog. Take over evening clean-up after dinner.

For the nausea. Identify foods she can keep down (often crackers, ginger, plain toast, fruit). Stock them. Don't cook anything strong-smelling near her. Bring her food to her if she's lying down. Keep a glass of water at her bedside. Have a discreet trash can or zip-top bag available for emergencies (cars, etc.). Skip alcohol around her for now (it can be off-putting).

For the mood shifts. Do not take it personally. Do not say "you are being hormonal" (even if you are right). Check in, briefly, frequently. Let her vent without trying to fix.

For the secret-keeping. Be careful with your own social-media presence. Do not tell anyone she has not okayed. Have a code phrase with her for "I do not want to drink at this event, find me an out."

For the fear (the loss-fear is real in the first trimester). Do not promise that nothing will go wrong. Hold her hand at the early scans. Listen when she shares anxiety. Do not tell her "to stop worrying."

Chapter 4: Trimester One, Your Own Experience

The first trimester is often disorienting for the non-birthing partner too. The pregnancy is invisible from the outside, but it is also invisible from your own life in a way. You may feel:

- Less excited than you expected (your bonding is not on the same timeline as a pregnant body).
- Anxious about the pregnancy itself.
- Anxious about your finances.
- Anxious about how this changes your career.
- Anxious about how this changes your relationship.
- Worried you will not be a good partner or parent.
- Worried that the loss-risk is keeping her quiet from you.
- Confused because nobody is asking how YOU are.
- Quietly grieving the just-the-two-of-us version of your life.

Who you can talk to about this:

- A friend who has been through it (different friend than usual maybe, the one who has had a baby).
- Your own therapist (yes, even now).
- A non-pregnant friend who will not get weird about it.
- Your partner, gently, when she has the energy.

A note: your emotional experience is not in competition with hers. Both of you can be struggling. Neither has to minimize.

Chapter 5: Trimester One, Appointments and Scans

What appointments you should be at:

- First confirmation visit (some people want partner there).
- First ultrasound (often around 6 to 10 weeks). This is one most partners want to be at. It is also a moment where, if there is bad news, you want to not be alone.
- Nuchal translucency / first-trimester genetic screening (if doing). The conversation about whether to test, what the results would mean, is a both-of-you conversation.

What you can do at appointments.

Drive. Be the second pair of ears. Ask the question she was too scared to ask. Take notes. Hold her hand during anything that hurts. Carry the bag.

What you should not do at appointments.

Take over the conversation with the provider. Speak for her. Interrupt her when she is talking. Treat the provider as if you are the patient. Be on your phone.

Chapter 6: Trimester Two, What Is Happening

For most pregnant people, the second trimester is the better one. Nausea often eases. Energy comes back. The body shows the pregnancy to the world.

Your partner is now often feeling better physically (not always, some carry first-trimester symptoms longer or develop new ones). Visibly pregnant to others. Receiving more comments, touches, opinions from strangers. Going through the anatomy scan and genetic results. Starting to think about logistics (nursery, leave, names). Sometimes feeling the baby move.

What is happening that you cannot see.

Organ development continues, refinement. The baby is now visible on ultrasound as a small human. Movement, often felt by the partner before others. Your partner is starting to bond more concretely.

Things that can still happen.

Anatomy-scan findings that change the conversation. Diagnoses for the baby that did not surface earlier. Pregnancy complications that develop in the second trimester (e.g. preterm-labor risk factors).

The second trimester is generally lower-risk, but "lower" is not "none." If something prompts you both to slow down, the team will tell you.

Chapter 7:

Trimester Two,

How to Be Useful

Physical. Help her find clothes that fit (maternity, second-hand, stretchy items). Help her find a comfortable sleep setup as the bump grows. Take walks together (movement helps for many people). Run errands she does not have energy for.

Social. Be ready for unsolicited comments at events. Practice a script with her ahead of time for stranger comments ("she doesn't want to talk about it," "that's not really our thing," etc.). Help her enforce her own limits when grandparents-to-be overstep. Take photos when she asks, do not take photos when she does not.

Logistical. Start showing up at the appointments. Co-research the choices that need to be made (birth setting, pediatrician, baby gear). Make a financial plan together. Plan parental leave for yourself if applicable.

Emotional. Notice when she is over-stimulated at a family gathering and offer to leave early together. Notice when she is enjoying something she did not expect to. Mark the small moments.



Chapter 8: Trimester Two, Your Own Bond

For non-birthing partners, the bond often arrives later than for the pregnant person. The pregnant person is interacting with a small human inside their body. You are interacting with a concept and some ultrasound images.

The bond often clicks at:

- The 20-week anatomy scan (the baby looks like a baby).
- The first time you feel a kick from outside.
- The first time you see the baby move on an ultrasound.
- The first contraction (suddenly it is real).
- The first day at the hospital.
- A moment in the first week that just happens.

If you are NOT feeling a bond yet, that is normal. The bond often arrives in the months after birth, not before.

A note: parenthood is a practice, not a feeling. Partners who do not feel overwhelmed love before birth often fall deeply into it in the first months. You are not failing.



Chapter 9: Trimester Two, Names and Bigger Conversations

The name conversation.

Some couples land on a name early. Some go in circles for months. Some do not decide until the baby is born.

The respectful way to do it:

- Both of you contribute names.
- Both of you have veto power.
- No name is "the family name we MUST use" unless both agree.
- Final answer is "we will both like saying this name for the rest of our lives."

The disrespectful way:

- One partner insists, the other concedes.
- Family pressure picks the name.
- The pregnant partner's preferences are dismissed.

Other conversations that belong in trimester two:

- Where the baby will sleep (room-share, separate room, bed-share considerations).
- Pediatrician selection.
- Childcare planning (if applicable to your situation).

- Whose family visits when.
 - The first month at home: who is doing what.
 - Money: what changes, what stays the same, what is the new baseline.
 - Wills, life insurance, beneficiaries (if you have not done this, now is the time).
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Chapter 10: Trimester Three, What Is Happening

This is the home stretch. Your partner is heavily pregnant (the bump is real, the discomfort is real). Sleeping poorly. Often unable to eat full meals. Anxious about delivery. Practical-mode about the house, the bag, the plan. Sometimes tearful for no clear reason. Sometimes overly emotional about commercials, news, the dog.

Things that are happening that you cannot see.

Baby has grown to near-full size. Body is preparing for labor in subtle ways (some of which the pregnant person can feel, like Braxton-Hicks). Hormones are shifting toward the delivery phase.

What to watch for:

- Sudden swelling, especially face and hands (preeclampsia warning).
- Severe headache or vision changes.
- Severe abdominal pain.
- Bleeding.
- Decreased baby movement (after week 28, this is the big one).
- Water breaking (often slow leak, sometimes gush).
- Regular contractions.
- Anything she calls you about with concern.

If any of these, call the provider. Your job is to call when she is in too much pain or distress to call herself.

Contact numbers on your phone:

- Her OB office (saved).
 - After-hours line (saved).
 - Hospital labor and delivery (saved).
 - Backup driver (in case you are not available).
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Chapter 11: Trimester Three, The "If Labor Starts" Plan

When she tells you it might be starting:

1. Stay calm. Most labors are NOT the "go right now" version.
2. Listen to what she is feeling.
3. Time the contractions if there are any.
4. Call the provider's after-hours line for guidance.

When it is definitely happening:

1. Bag.
2. Car seat.
3. Phone.
4. The list of who to call.
5. Lock the door.
6. Go.

During labor, for you specifically.

Be the calm voice. Be the second pair of ears for the team. Hold her hand. Get her ice chips, water, whatever they allow. Bring her food after, when it's allowed. Photograph what she has asked you to photograph. Do NOT photograph what she has asked you not to. Read the birth plan to remind yourself of her preferences. Be ready to advocate for her if she is too tired.

What to say during the hardest part:

- "You're doing great."
- "I am with you."
- "Breathe."
- "I am right here."
- Whatever she has previously told you she wants to hear.

What not to say during the hardest part:

- "I know how you feel." (You don't.)
- "Just breathe." (Too generic.)
- "Almost done." (Often not true.)
- Anything that minimizes what she is experiencing.

After delivery.

Photograph the baby with her, if she wants. Photograph her without her permission only with her enthusiastic yes. Call the family on her behalf. Bring her real food when she's hungry. Sit with her. Cry if you need to. Notice what you want to remember about this day.

Chapter 12: The Last Week Before the Baby

This is one of the strangest weeks of your life.

You may feel: strangely calm, strangely panicky, sad that the just-the-two-of-you era is ending, excited, restless, tearful, hyper-organized, frozen, all of these in the same evening.

Things to do in the last week:

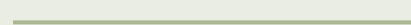
- A meal out together (it may be your last for a while).
- A movie at home together.
- A walk together.
- Tell your partner one thing you are grateful for about her before this chapter closes.
- Write down a small note for the version of you who is in the delivery room next week.
- Read the birth plan together one more time.
- Sleep.
- Eat the food you have been craving.

Chapter 13: Your Own Grief and Fear

This chapter is for you. Most partners do not give themselves time for this.

What you are scared of. What you are grieving about your old life. What you are scared you will not be good at. What you are scared of losing in your relationship. What you want for yourself, in this new chapter. Who you could talk to about this, besides your partner. The therapist you might see (yes, this is on the table for you too).

A reminder: you are allowed to have feelings that are not about the pregnant person. Your feelings are not selfish. They are the cost of loving someone and building a life with them. The feelings deserve a place to live, even if that place is not your partner.



Chapter 14: Things to Say + Things Not to Say

A reference chapter. Re-read periodically.

Things to say often:

- "You are doing the hardest job in this house."
- "I am with you."
- "What do you need right now."
- "Tell me how I can help."
- "I am proud of how you are handling this."
- "I love you."
- "I am not going anywhere."
- "I will figure out [task]."
- "I noticed [specific thing she did or felt]."
- "Whatever the birth looks like, you and I are still us."

Things to never say:

- "You're being hormonal."
- "Calm down."
- "Other women have done this."
- "My mother did this with no help."
- "You're overreacting."
- "Why are you crying?"
- "Just relax."
- "It can't be that bad."

- "I'm tired too."
- "When will you be back to normal?"

Things you can say that are worth practicing:

- "I notice you're frustrated. Can you tell me what would help?" (vs "Why are you so mad?")
- "Let me handle that, you sit." (vs "Are you sure you can't do it?")
- "I'll take care of dinner." (vs "What do you want for dinner?")
- "Tell me what you're afraid of about the birth." (vs "Don't worry about it.")

A note: words matter. The exhausted pregnant person remembers what you said and what you did not. This is not unfair. This is how emotional labor works. The bank account of trust is built word by word.

Chapter 15: Your Relationship Is Changing

The marriage / partnership is going through one of the largest stress tests it will ever see.

What is changing.

You used to plan around two people. Now you are planning around three (or four). You used to fight (or not fight) over small things. Now the small things have higher stakes. You used to have time. Now time is going to compress, and you need a strategy. You used to be peers. The pregnant person is now in a temporary medical and physical role you cannot share, and that asymmetry is real.

What to do during pregnancy to keep the partnership strong:

- Continue having a weekly conversation about you-as-a-couple, not just the pregnancy.
- One date night per month at minimum (even if it is at home).
- Physical affection that is not sex (especially when energy and body changes affect sex).
- Express appreciation specifically, not vaguely.
- Have a fight, in a healthy way, when one is needed (do not bottle it for the pregnancy).

What to consider starting now:

- Couples counseling, even just a few sessions, to set good habits for the first year.
 - A read-this-together book that you both engage with.
 - A weekly "state of the partnership" 15-minute conversation (yes, scheduled).
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Chapter 16: Partner Mental Health

A chapter that exists because partners are not always asked.

How you are, honestly. How much you are sleeping. Whether you are eating regular meals. What you are drinking (alcohol especially). When you last laughed. When you last cried, if you have. How your work is going. How your friendships are.

Any of these true for you:

- I have been more anxious than usual.
- I have been sadder than usual.
- I have been drinking more than usual.
- I have been withdrawing from people.
- I have been having thoughts that worry me.
- I am not feeling excited about the baby in a way that bothers me.
- I do not feel like myself.

If two or more of these are true, that is information. Partner mental health during pregnancy is real and underdiscussed. A therapist is a reasonable next step.

A note: getting your own mental-health support now is one of the kindest things you can do for your partner and for the baby. A well-regulated adult in the home is what this family is going to need. Starting with yourself is not selfish. It is the foundation.

Chapter 17: A Final Letter

You read this book.

You read it because you wanted to be more than a partner who shows up at the hospital with a phone charger. You wanted to be the one who knows what is happening, knows what to do, knows what not to say.

That is the kind of partner who makes pregnancy meaningfully easier for the person doing the physical work. It is also the kind of partner who arrives at the delivery room ready, not panicked, not unprepared.

Your role does not look like hers. It is not the same job. But it is still a job. The book is the playbook.

You did not have to read this. You did. That is the thing your partner will remember.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents.

The printable companion to this book is the **Partner-During-Pregnancy Companion** at notes.soothemade.com.

Also from Soothemade

- **The Partner's Postpartum Playbook**, the next book in the series, for after the baby arrives.
 - **The Birth Plan Pack**, the document you and your partner will write together.
 - **The Birth Anxiety Workbook**, for the fear that may be hers and yours.
 - **The Pregnancy + Work Planner**, for navigating leave and accommodations at work.
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Crisis lines

United States

- 988, Suicide and Crisis Lifeline
- 1-833-852-6262, Postpartum Support International HelpLine (also for partners)
- 1-833-943-5746, Maternal Mental Health Hotline

United Kingdom

- 116 123, Samaritans

Canada

- 9-8-8, Suicide Crisis Helpline

Australia

- 1300 726 306, PANDA

This book is a planning and support tool, not medical advice.

A Soothemade book. Made slowly, in plain language.